

COMMENTARY

Different Disciplines Have Different Perspectives, So What Are the Implications for the Multidisciplinary Field of Alcohol Public Health Science? A Reply to “All Boundaries Have Two Sides” by Bray and Kenkel (2024)

IN THEIR COMMENTARY in this issue of the *Journal of Studies on Alcohol and Drugs*, Bray and Kenkel (2024) write, “It would be unfortunate if readers came away with nothing more than a distrust of Jon Nelson’s work” (p. 428). With that, we wholeheartedly agree. The key message of our article (Bartlett & McCambridge, 2024) is that boundaries between disciplines, or fields of study, are boundaries between communities of science, and we draw attention to the concept of boundary work and the nature of the work involved, the early stage of explicit normative debates about these issues in alcohol science, and the need for alcohol public health science in particular to make progress on these issues due to targeting by the alcohol industry. These boundaries are real things—they are “social facts” (Durkheim, 1982)—and they are partially constituted by differences in the value systems of the communities in question. The answers to the questions of “what it is to do good science” and “what it is to behave as a good scientist” differ between communities on normative as well as methodological grounds (Cetina, 1999). In this spirit, we are thankful and do respect this articulation of the thinking of health economists. We also welcome the debate for reasons previously given.

Boundaries are not really self-imposed; they are generated by understandable social processes. Bray and Kenkel’s commentary on our article illustrates this well, providing us with a neat sketch of the way in which the normative system of economics as a community differs from that of public health research. Economists, as members of that community, value different things, which shapes the scientific questions they ask, the methods that they use, and the conclusions they draw. Bray and Kenkel (2024) write, “Economists, however, are concerned about possible loss of consumer sovereignty” (p. 427), and “many, if not most, economists have been trained to value the free market and to be distrustful of governmental interventions in markets,” generating “economist’s inherent distrust of pricing interventions” (p. 428). The differences illustrated by these normative distinctions are also expressed in the articulated

thinking about conflicts of interest. Economists take a more laissez-faire approach to funding from the tobacco or alcohol industry than do public health researchers from other disciplines, where the evolution of norms was discussed in our article.

To this extent, we are aligned with Bray and Kenkel. Our descriptions of what “is” are largely compatible. We might question how far the views presented on consumer sovereignty, for example, actually represent the discipline as described, as clearly not all economists, and particularly public policy economists, view the world through the prism of market-based logics. However, that would really be a question asked in ignorance, from well outside the discipline. We also make the point in our article that emerging alcohol public health norms are more implicit than explicit, so it would be incorrect to ascribe to them qualities of being well formed or agreed. Here we draw on insider knowledge and will make different kinds of claims below. By extension, we clearly differ from the commentators on what “ought.” Bray and Kenkel see these boundaries as obstacles to what they describe as good and best science and policy. We regard that as wishful thinking. There are reasons different disciplines have developed as they have, and these differences need to be reckoned with explicitly in multidisciplinary fields such as alcohol science more broadly and alcohol public health science in particular.

A key challenge for policymakers is to differentiate between competing knowledge claims. They must do so without having contributory expertise of the technical domain, so they cannot be arbiters of scientific disputes on technical grounds (Collins & Evans, 2019). This includes handling claims made in relation to scientific controversies, including “counterfeit” controversies (Bartlett & McCambridge, 2021; Weinel, 2019). Scientists giving attention to norms around conflict of interest help policymakers understand the social position and functions of particular claims (Babor & Robaina, 2013). Which claims are taken seriously by a scientific community, and are thus added to the community’s collective knowledge, are determined by social as well as technical factors. The more

this kind of knowledge is accessible to policymakers, the more usable are the scientific outputs in policymaking.

These kinds of issues really matter in fields that are targeted by powerful commercial forces. Tobacco and alcohol are prime examples (Francey & Chapman, 2000; McCambridge et al., 2022), and this is why we are more sharply critical of the second part of the commentary on conflicts of interest. This material does not seem to seek to make scientific arguments as such; it is naive in the sense of not locating the comments made in relation to the literature on which we based our study, or indeed any relevant scientific literature. Any reader would not know from this that there have been long-term projects to shape science for public relations purposes for both tobacco (Proctor, 2012) and alcohol (McCambridge et al., 2021), for example, and indeed that they grew up together, with alcohol inheriting from tobacco (Bartlett et al., 2023). In our view, this makes the equation of the vested interests in science of industry and government or public health agencies somewhat dangerously ill-informed. Both do have interests, but they are in opposition to each other; government or public health agencies generally seek to promote science in the public interest, whereas these industries generally seek to undermine science for business interests (Stenius & Babor, 2010). There are, of course, counterexamples on both sides, such as the use of suppression clauses by governmental research commissioners who seek control over the outcomes (McCrabb et al., 2021). If there are examples of “the research of other disciplines that have found a way to exploit” alcohol industry funding, we would be very interested to know what these are, and to evaluate them, as there may be unintended consequences to be identified.

In our article we note that few of Nelson’s contributions to the scientific literature are in alcohol or public health journals. Nelson’s work is rarely discussed within this community as a credible scientific contribution. A policy-maker might look at this body of work or that produced in similar circumstances, because such work does get promoted in public policy consultations (Bartlett & McCambridge, 2022), and think, “perhaps public health researchers are wrong.” We suggest that it is more appropriate to regard Nelson’s work as being at odds with the consensus within the alcohol public health research field and coming from outside it. We make no claims about the personal motivations of Nelson in conducting and promoting this work. Our primary focus has been on the functions this work serves for the alcohol industry—the funders of this research—and the need for our community to think through and discuss how to respond.

This is a far from trivial undertaking. It is full of peril because it raises challenging issues that are undeniably difficult to discuss (McCambridge & Mitchell, 2022; Mitchell & McCambridge, 2021). We regard it also as essential to the development of our multidisciplinary field.

A good starting point is finding agreement in debates also characterized by disagreements. We end by agreeing with Bray and Kenkel (2024) when they say that economists “are good targets for industry funding. It may be that the industry need only stimulate some interest in the topic of alcohol control policy and then let an economist’s research go where it naturally will” (p. 428). We welcome further contributions from these authors, other economists and other disciplines that give attention to the implications of this statement, and other issues raised in this debate on the development of the multidisciplinary field of alcohol public health research more broadly.

As we proceed, let us observe high standards in declarations of conflicts of interest (Babor & Miller, 2014; Society for the Study of Addiction, 2008), which do not of course imply that the writer has nothing useful or important to contribute, just that making such declarations is the norm for good reasons. One major driver of this debate within the scientific community can be to make science more useful to public health, and wider public policy, by making all our claims more open to examination. Better science is another.

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